

Pembrolizumab + pemetrexed + carboplatin

Lung · NCCP 00568 · version 5

My treatment

Cycles 1–4: pembrolizumab + pemetrexed + carboplatin every 21 days.

Maintenance: pembrolizumab + pemetrexed every 21 days if treatment continues.

How to use this passport

Record the symptom you actually felt, not just the organ it might come from. Use the simple severity guide on page 2 and bring the passport to reviews.

SYMPTOMS TO WATCH FOR

These are the same symptoms highlighted in the visual anatomy guide, translated into patient language.

Symptom	Main medicine link	What to notice
Shortness of breath / new cough	Pembrolizumab	New breathlessness, worsening cough or breathing that feels harder than usual.
Diarrhoea / bowel change	Pembrolizumab / chemotherapy	Loose or more frequent stools, tummy pain, blood or mucus, or difficulty keeping fluids up.
Fatigue / new weakness	Pembrolizumab / chemotherapy	Tiredness that is new or clearly worse, especially if rest does not help or normal activities become difficult.
Rash / skin changes	Pembrolizumab / chemotherapy	New rash, itch, blistering, skin pain or mouth/eye involvement.
Numbness / tingling / weakness	Pembrolizumab	New numbness, tingling, muscle weakness or difficulty using hands, walking or standing.
Vomiting	Pemetrexed / carboplatin	Repeated vomiting, inability to keep fluids down or vomiting despite anti-sickness medicines.

Do not wait to grade these red flags

Fever / shivers / suddenly unwell, chest pain or fainting, yellow eyes/skin or dark urine, severe headache/confusion/seizure, or markedly reduced urine/new swelling: contact oncology urgently. Severe breathing difficulty, collapse, seizure or severe chest pain: call 112/999.

Mon–Fri 8am–5pm

Acute Oncology Service
087 181 9645

Sat / Sun / Bank Holiday 9am–5pm

021 4922000
Ask for the medical oncology team.

Evenings / overnight

GP out-of-hours or nearest ED.
Emergency 112 / 999

Treatment portal

<https://sactcheck.com/patient/00568/>

The QR contains regimen information only, not patient-identifying data.



Contact details from the CUH Medical Oncology patient leaflet. This passport is a communication aid and does not replace your oncology team's advice.

Describe what you feel, then use the grade that fits best

Patient-facing wording informed by NCI CTCAE v6.0 severity concepts. Your clinical team makes the formal CTCAE grade.

The table deliberately uses symptoms rather than organs. If more than one description fits, use the more severe description and contact your team if worried.

- Grade 0 · none
- Grade 1 · mild
- Grade 2 · moderate
- Grade 3 · severe / urgent
- Grade 4 · life-threatening / emergency

Symptom	Grade 0	Grade 1	Grade 2	Grade 3	Grade 4 / emergency
Shortness of breath	No breathlessness.	Breathless only with more strenuous activity such as brisk walking or stairs.	Breathless with light activity or usual daily tasks; may need to stop and rest.	Breathless at rest or with very little activity; washing, dressing or self-care is difficult. Urgent same-day assessment.	Severe breathing difficulty / life-threatening breathing problem. Call 112/999.
Diarrhoea	Your usual bowel pattern.	Looser or more frequent stools than usual, but manageable.	About 4–6 extra stools in 24 hours or bowel symptoms affecting normal daily activities.	7 or more extra stools in 24 hours, severe symptoms, dehydration risk, or difficulty looking after yourself. Urgent assessment.	Life-threatening dehydration or other severe complication. Emergency assessment.
Fatigue	Usual energy.	More tired than usual but improves with rest.	Tiredness does not fully improve with rest and makes usual household/work tasks harder.	Tiredness makes washing, dressing, eating or other self-care difficult.	CTCAE fatigue does not use Grade 4. Sudden collapse or severe new weakness is an urgent problem.
Numbness / tingling	None.	Mild tingling or numbness without limiting activities.	Symptoms interfere with tasks such as buttons, writing, cooking or other normal activities.	Symptoms interfere with walking, dressing, washing or other self-care.	Rare life-threatening neurological problem / urgent intervention required.
Rash / skin changes	No new rash.	Small or mild rash / itch with little effect on daily life.	More widespread or persistent rash / itch affecting normal activities.	Extensive, painful or severe rash, or skin symptoms limiting self-care. Urgent assessment.	Widespread blistering/peeling, mouth or eye involvement, or other life-threatening skin reaction: emergency assessment.
Vomiting	None.	1–2 episodes in 24 hours; able to drink.	3–5 episodes in 24 hours or struggling to maintain normal intake.	6 or more episodes, cannot keep fluids down, or IV fluids / hospital assessment may be needed.	Life-threatening consequences such as severe dehydration or collapse: emergency assessment.

Do not wait to grade these red flags

Fever / shivers / suddenly unwell, chest pain or fainting, yellow eyes/skin or dark urine, severe headache/confusion/seizure, or markedly reduced urine/new swelling: contact oncology urgently. Severe breathing difficulty, collapse, seizure or severe chest pain: call 112/999.

Reference: NCI Common Terminology Criteria for Adverse Events (CTCAE) v6.0. Wording has been simplified for patients and should not be used as a substitute for formal clinician grading. [CTCAE v6 source](#)

Cycle-by-cycle symptom record · C1 to C4

Write a grade 0–4 for graded symptoms, or a brief note. Red-flag rows are for recording only — do not delay contact while deciding on a grade.

Symptom	C1	C2	C3	C4
Shortness of breath / new cough				
Diarrhoea / bowel change				
Fatigue / new weakness				
Rash / skin changes				
Numbness / tingling / weakness				
Vomiting				
Fever / shivers / suddenly unwell				
Chest pain / palpitations / fainting				
Yellow eyes/skin / dark urine				
Reduced urine / new swelling				
Other new symptom				

CYCLE DATES / TREATMENT CHANGES

C1 / C2 dates and notes

C3 / C4 dates and notes

Carboplatin is part of induction cycles 1–4 in the standard regimen plan. Treatment may be delayed, modified or stopped based on clinical review.

Cycle-by-cycle symptom record · maintenance

Maintenance usually continues with pembrolizumab + pemetrexed every 21 days if clinically appropriate.

MAINTENANCE CYCLES M1-M4

Symptom	M1	M2	M3	M4
Shortness of breath / new cough				
Diarrhoea / bowel change				
Fatigue / new weakness				
Rash / skin changes				
Numbness / tingling / weakness				
Vomiting				
Fever / shivers / suddenly unwell				
Chest pain / palpitations / fainting				
Yellow eyes/skin / dark urine				
Reduced urine / new swelling				
Other new symptom				

Cycle-by-cycle symptom record · M5 to M8

Use the same patient-language grades from page 2. Red-flag symptoms should prompt contact rather than delayed self-grading.

MAINTENANCE CYCLES M5–M8

Symptom	M5	M6	M7	M8
Shortness of breath / new cough				
Diarrhoea / bowel change				
Fatigue / new weakness				
Rash / skin changes				
Numbness / tingling / weakness				
Vomiting				
Fever / shivers / suddenly unwell				
Chest pain / palpitations / fainting				
Yellow eyes/skin / dark urine				
Reduced urine / new swelling				
Other new symptom				

Need more cycles? Continue on a fresh copy of this page or record additional cycles in the live patient portal / clinic notes.